

Media Information

09-05-2025 18:35 Datavant Fax Id: 6/14

RUCKER, Derrica (id)
MRI BREAST BILATERAL (RT/LT) WO/W CONTRAST (#515831649, Final, 9:30am)



Summit Health
Part of VillageMD*

Summit Health
574 Springfield
Westfield NJ
908.389.6416

Patient Name:	DERRICA RUCKER	Date of Birth:	
Patient MRN:	20740116	Accession Number:	5158316457
Referring Doctor:	WILLIAM DIEHL	Exam Date:	09/26/2025

MRI BREAST BILATERAL (RT/LT) WO/W CONTRAST

CLINICAL HISTORY: Highly suspicious imaging findings in the left breast and left axilla. Patient underwent ultrasound-guided biopsy in the left breast and left axilla on 8/26/2025. Pathology is pending.

TECHNIQUE: Bilateral breast MRI was obtained using standard protocol. Multiple sequences were obtained in the axial and sagittal planes, including pre and post contrast, with and without fat suppression. Patient received 26 ml of Dotarem. Images were reviewed using CADstream, allowing 3D imaging in several planes, subtraction imaging and maximum intensity projection with color to assess kinetics.

COMPARISON: No prior breast MRI examinations are available for comparison. Correlation was made with prior relevant mammogram and sonogram from outside institution.

DENSITY: There is scattered fibroglandular tissue.

FINDINGS:

There is mild background parenchymal enhancement.

There is highly suspicious irregular enhancing and likely partially necrotic vascular mass in the upper inner/upper central left breast, measuring about 4-4.5 cm. There is associated biopsy marker clip. This was recently biopsied. Pathology is pending at this time.

There is left axillary lymphadenopathy. The largest morphologically abnormal left axillary lymph node measures 3 cm and contains biopsy marker clip. Pathology is pending at this time.

There are no suspicious lesions in the contralateral right breast.

There is no right axillary lymphadenopathy.

There is no internal mammary chain lymphadenopathy.

There is no abnormal chest wall or skin enhancement.

IMPRESSION:

1. Recently biopsied highly suspicious enhancing mass in the upper inner/upper central left breast. There is associated biopsy clip. Pathology is pending at this time. Patient will follow-up with the breast surgeon and oncologist regarding the pathology results and further management.

Interpreted by a Board Certified Radiologist at Summit Health
973.404.9888

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2. Left axillary lymphadenopathy - The largest 3 cm morphologically abnormal left axillary lymph node contains biopsy marker clip. Pathology is pending at this time. Patient will follow-up with the breast surgeon and oncologist regarding further management.

3. No suspicious findings in the contralateral right breast.

BI-RADS Category 5: Highly Suggestive for Malignancy

Electronically signed by Maryna Koznetsov, M.D. (Aug 28, 2025 14:49:34)

Interpreted by a Board Certified Radiologist at Suncoast Health
973.404.5888

████████ 00:00

Attached To:

OUTSIDE RADIOLOGY ORDER ██████████

Orders Only on ██████████ with Historical Provider, ██████████

Source Information

Jaslee Quinones | Msk Pas